Adjustment Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

An adjustment disorder is a stronger-than-expected emotional or behavioral reaction to an identifiable life stressor, such as a breakup, job loss, illness, move, or financial crisis, that causes significant distress or impairment. In DSM-5-TR it sits in "Trauma- and Stressor-Related Disorders" (American Psychiatric Association [APA], 2022).

Common but under-researched: adjustment disorders are among the diagnoses most often made in everyday practice, yet they lack specific symptom criteria and are hard to tell apart from normal stress reactions on one side and depression or PTSD on the other (Casey & Bailey, 2011; Bachem & Casey, 2018). In palliative-care settings, 15.4% of patients met criteria (Mitchell et al., 2011).

Counselor framing: adjustment disorders are usually time-limited and respond to brief, practical help. They still deserve attention: suffering is real, suicidality can occur, and some cases turn into more serious disorders (O'Donnell et al., 2019).

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Onset:** emotional or behavioral symptoms begin **within 3 months** of an identifiable stressor.
- **Significance:** distress out of proportion to the stressor (taking culture into account) and/or significant impairment in social, work, or other functioning.
- **Exclusions:** does not meet criteria for another disorder and is not just a flare of an existing one; is not normal grief or prolonged grief disorder.
- **Course:** once the stressor or its consequences end, symptoms resolve **within 6 months**. Specify acute (under 6 months) or persistent (6 months or more, for ongoing stressors).
- **Subtypes:** with depressed mood; with anxiety; with mixed anxiety and depressed mood; with disturbance of conduct; with mixed disturbance of emotions and conduct; unspecified.

Differential diagnosis (general clinical consensus): if full criteria for major depression, an anxiety disorder, or PTSD are met, diagnose that disorder instead. If the stressor was a death, consider normal grief and prolonged grief disorder.

Suicide risk: among hospital patients referred to liaison psychiatry, those with adjustment disorder showed suicidal behavior at **lower** levels of depressive symptoms than those with a depressive episode (Casey et al., 2015). Do not assume "only an adjustment disorder" means low risk; ask directly.

When counselors must refer (general clinical consensus): refer for psychiatric evaluation for suicidality, substance misuse, or symptoms that worsen or persist beyond 6 months after the stressor ends.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in adjustment disorders. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
K10 (Kessler Psychological Distress Scale)	General psychological distress; quick screen in any setting.	Self-report, 10 items	No (free)	None beyond general clinical training	Original article
PSS (Perceived Stress Scale)	How stressful and overwhelming life feels.	Self-report, 10 or 14 items	No (free for non-commercial use)	None beyond general clinical training	Original article

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PHQ-9 (Patient Health Questionnaire-9)	Screening and tracking depression; item 9 screens for suicidal thoughts.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
GAD-7 (Generalized Anxiety Disorder-7)	Screening and tracking generalized anxiety.	Self-report, 7 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
PCL-5 (PTSD Checklist for DSM-5)	Checking for PTSD when the stressor was traumatic.	Self-report, 20 items, 5 to 10 minutes	No (free)	None beyond general clinical training	National Center for PTSD
WHODAS 2.0 (WHO Disability Assessment Schedule)	How much symptoms interfere with daily life and functioning.	Self- or proxy-report, 36 items	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening and assessing suicide risk; brief screener and full versions.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: brief CBT, delivered in person or online, has the best (though still limited) evidence. There is little support for routine medication. Practical problem-solving and a gradual return to normal activities help.

Evidence overview (honest findings):

- **Overall:** a 2025 meta-analysis of 16 trials (3,027 participants) found that in-person and internet-based CBT may both be effective (Cowansage et al., 2025). An earlier review of 29 studies rated the quality of evidence for every psychological and drug treatment as low to very low (O'Donnell et al., 2018).
- **Brief CBT:** among 120 patients with adjustment disorder with anxiety, a 5-week CBT program, given face-to-face or as blended online-plus-nurse sessions, reduced anxiety, depression, worry, and stress versus a wait-list, with gains mostly kept at 6 months (Leterme et al., 2020).
- **Self-help:** a 1-month CBT self-help manual for burglary victims reduced preoccupation and posttraumatic symptoms compared with a wait-list (d = 0.17 to 0.67), and 87% engaged with it (Bachem & Maercker, 2016).
- **Work-related stress:** an activating, problem-solving intervention through occupational physicians returned people to work sooner and with fewer relapses, though symptoms improved equally in both groups (van der Klink et al., 2003).
- **Medication:** a review of 23 studies found a wide variety of medications and therapies tested in small, mixed studies (Constantin et al., 2020); there is no strong evidence for routine drug treatment (O'Donnell et al., 2018).

CBT for adjustment disorders: how it is used (Bachem & Maercker, 2016; Leterme et al., 2020):

- **Normalize** the reaction and explain the link between the stressor, thoughts, feelings, and behavior.
- **Identify and challenge** catastrophic or helpless beliefs about the stressor: "My life is ruined," "I can't handle this," "Nothing will ever be the same."
- **Problem-solve** the practical consequences, plan a **graded return** to activities and work, and build relaxation and coping skills (van der Klink et al., 2003).

5. Religion, spirituality, and Christian integration: what the evidence shows

Faith can buffer stress. Among 348 hospital patients with depressive episodes or adjustment disorder, organized religious activity, private religious activity, and intrinsic religiousness each **weakened the link between stressful life events and depressive symptoms**, including 6 months later (Lorenz et al., 2019).

How faith is used matters. Across 49 studies, positive religious coping (seeking God's help, finding spiritual meaning) was linked to better adjustment to stress, and negative religious coping (feeling punished or abandoned by God) to worse (Ano & Vasconcelles, 2005).

Intervention studies. We found **no trial of Christian or religiously integrated treatment for adjustment disorders** (PubMed search, October 31, 2026).

Honest summary for teaching:

1. Brief CBT, problem-solving, and support are the evidence base, though the research is still limited. Faith integration supports them; it does not replace them.
2. Religious involvement and positive religious coping appear to buffer the effect of stressful events; negative religious coping does the opposite.
3. No faith-integrated trial exists. Because adjustment disorders are brief and stressor-focused, they suit a short, faith-informed CBT approach, but this has not been tested.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within brief CBT. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Ask how the person interprets the stressor spiritually. Listen for negative religious coping: "God is punishing me with this," "God has forgotten me," or "A real Christian wouldn't be this upset" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Test catastrophic predictions against the evidence and the person's past coping, and replace helplessness with concrete problem-solving steps (Bachem & Maercker, 2016).
- **Replace (pastoral).** Offer Scripture of God's presence and help in hard seasons:
  - "God is our refuge and strength, an ever-present help in times of trouble." (Psalm 46:1)
  - "In the world you will have tribulation. But take courage; I have overcome the world!" (John 16:33). Trouble is not a sign that God has left.
  - "Be anxious for nothing, but in everything, by prayer and petition, with thanksgiving, present your requests to God. And the peace of God… will guard your hearts and your minds in Christ Jesus." (Philippians 4:6–7)
  - "Behold, I am about to do something new… I will make a way in the wilderness and streams in the desert." (Isaiah 43:19). Hope for life after the change.
- **Faith community (clinical).** Encourage continued participation in worship and church fellowship during the stressful season, since organized and private religious activity both buffered stress (Lorenz et al., 2019).
- **Ministers:** offer practical help and presence, and refer if there is suicidal thinking or if distress lasts beyond 6 months after the stressor ends.

7. Quick classroom takeaways

- Symptoms start within 3 months of a stressor, are out of proportion or impairing, and resolve within 6 months after it ends. Diagnose depression, anxiety, or PTSD instead if their criteria are met.
- Brief CBT (in person or online) has the best, though limited, evidence; routine medication is not supported. Do not overlook suicide risk.
- Faith buffers stress. Challenge "God is punishing me" and "I can't handle this," and replace them with the truth of God's presence and help.
- Use brief free measures (K10, PHQ-9, GAD-7) to track distress, and the PCL-5 when a traumatic stressor may point to PTSD instead.

Sources note

All 14 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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